

MATERNITY BIRTH PLAN & PREFERENCES

Labor, Delivery, & Immediate Postpartum Preferences

Patient Name:	Jane Doe	DOB:	05/12/1992
Primary OB-GYN:	Dr. Sarah Jenkins, MD	Estimated Due Date:	02/13/2027
Support Person / Partner:	John Doe (Husband)	Doula Name (if applicable):	Emily Smith, CD(DONA)

1. LABOR ENVIRONMENT PREFERENCES

- **Lighting & Ambience:** Dim lights in the labor suite, peaceful atmosphere with a custom relaxation playlist.
- **Visitors:** Limited to primary support person (partner) and certified doula only. No extra visitors during active labor.
- **Mobility:** Freedom to move, walk around, and use a birthing ball/peanut ball during early labor.

2. PAIN MANAGEMENT PREFERENCES

- **Primary Approach:** Open to Epidural analgesia when requested during active labor.
- **Interim Coping Measures:** Warm shower therapy, deep breathing techniques, massage, and hydrotherapy prior to epidural placement.

3. DELIVERY & IMMEDIATE POSTPARTUM CARE

- **Delivery Position:** Semi-upright or side-lying position preferred.
- **Cord Clamping:** Delayed cord clamping requested (minimum 60–90 seconds or until cord stops pulsating).
- **Immediate Contact:** Immediate uninterrupted skin-to-skin contact for at least 1 hour post-birth.
- **Newborn Procedures:** Perform routine newborn evaluations (Apgar, weight, eye ointment, Vitamin K) while baby remains skin-to-skin on mother's chest whenever medically stable.

4. EMERGENCY CONTINGENCY PLAN (UNPLANNED C-SECTION)

- **Support Person in OR:** Partner to remain present by mother's head at all times during regional anesthesia.
- **Clear Screen Option:** Request a clear or drop-down drape at the moment of delivery if feasible.
- **Post-Op Skin-to-Skin:** Initiate skin-to-skin contact immediately in the operating room or recovery suite as soon as medically stable.